A FIELD GUIDE FROM PERSEVERANT DIGITAL · FOR ELECTIVE HEALTHCARE

How to Fill Your Consult Calendar With Patients Ready to Book

Stop paying for leads that never sit in the chair. A plain plan to get named when patients research, answer in minutes and turn more consults into accepted treatment.

THE SHORT ANSWER · 63 WORDS REVIEWED OCTOBER 15, 2026

Fill your consult calendar by fixing the path after the inquiry first: answer every request within minutes with a real person, confirm consults the way patients actually show up, and follow up on every unaccepted quote. Then publish honest cost and candidacy answers so Google and AI assistants name you, and judge each source by cost per accepted treatment, never cost per lead.

Your Meta leads look cheap. Then your coordinator calls and gets voicemail, or someone who doesn't remember the form. The few who book don't all show. The ones who do get a quote, say they need to think about it, and nobody calls them again.

Meanwhile the patients who will book research for weeks. They compare practices and ask ChatGPT what full-arch implants or a facelift should cost in your city. By the time they call, many have already narrowed their list.

Demand is there, and so is room on the schedule. ASPS reported on September 1, 2026 that cosmetic surgical procedure volume rose 7% in 2025 over 2024. The ADA Health Policy Institute reported in October 2025 that capacity is opening up and dentists are less busy, so the practices that convert inquiries win. The gap sits between the click and the accepted treatment plan. Here is how to close it.

Which lead sources actually produce accepted treatment?

Judge every source by what it costs to get one accepted treatment plan, not one lead. A $40 lead that never books costs more than a $200 lead that becomes a full-arch case.

Fill in the last 90 days from your practice management system, as totals by source, so no patient detail leaves it.

Source	Spend	Inquiries	Consults booked	Consults held	Treatment accepted	Cost per accepted treatment
Google search ads						
Google Business Profile						
Meta ads						
Website and organic search						
Doctor and patient referrals						
Seminars or open houses						

Cost per accepted treatment = spend ÷ treatment accepted

Example numbers, not benchmarks: $6,000 on Meta buys 150 leads at $40. Thirty book, 18 show, 4 accept: $1,500 per accepted treatment. The same $6,000 on Google search buys 40 leads at $150. Twenty book, 16 show, 6 accept: $1,000. The “expensive” channel wins.

If the source column is mostly blank, that's your first fix: ask and record the source on every phone booking.

Why do cheap Meta leads stop booking?

Cheap leads stop booking when the form asks too little and nobody calls fast. Meta also changed the rules. Search Engine Land reported on March 11, 2025 that Meta's restrictions on health and wellness advertisers can block optimizing toward lower-funnel events such as scheduling, with in-app lead forms affected least. So Meta may end up finding people who fill out forms rather than people who book.

What you control:

- [ ] Add one or two honest questions to the form, such as "When are you hoping to start?" and "Best time to reach you?" Never ask about conditions, diagnoses or treatment history.
- [ ] Offer a low-pressure next step, such as a cost and financing explainer or a recovery guide.
- [ ] Keep ad copy free of promised results. No "perfect smile in one day" and no implied before-and-after outcomes.
- [ ] Never build audiences from patient lists, and never send anything that could be health information back to Meta.

Treat Meta as an introduction. The booking happens later, through follow-up and search.

How fast should we call back a new inquiry?

Call within five minutes during open hours, by a person, and log every attempt. Harvard Business Review's "The Short Life of Online Sales Leads" (March 2011) audited 2,241 US companies and found only 37% responded to a web lead within an hour, with an average response of 42 hours. In a separate data set, firms that made contact within an hour were nearly seven times as likely to qualify the lead as those that waited longer.

Your front desk is checking patients in and answering insurance questions, so web inquiries wait. Fix that with a routine, not a new tool.

- [ ] One named person owns new inquiries each shift, with a backup.
- [ ] New leads trigger a phone alert, not just an email to a shared inbox.

- [ ] Call first. If no answer, send a short text right away and try again within the hour.
- [ ] After hours, an automatic text confirms you got the request and says when someone will call. A human calls at opening.
- [ ] Every text and email names the time and the practice, never the procedure.
- [ ] Texts go only to people who gave consent, and a “STOP” reply ends texting for that number.

On texting: the TCPA still requires prior express written consent for automated marketing texts. Since April 11, 2025, patients can revoke consent by any reasonable means, such as replying STOP, and you must honor it within 10 business days. This is not legal advice.

How do we cut consult no-shows?

Cut no-shows with a personal confirmation, clear reminders and an easy way to reschedule.

When	What	Wording that keeps patient data out
At booking	Call or text confirmation	“You’re booked with Dr. Lee on Tuesday at 2 p.m. at our Westlake office.”
3 days before	What to expect	“Your consultation takes about 45 minutes. Bring a photo ID and any questions.”
1 day before	Reminder with easy reschedule	“See you tomorrow at 2 p.m. Reply or call to move your time.”
2 hours before	Short nudge with parking or directions	“Parking is behind the building. Front desk will meet you.”
Missed consult	Same-day call, then a text	“We missed you today. Want to pick a new time?”

Never name the procedure in a reminder. A missed consult often means a schedule conflict, not a lost patient, so call the same day.

What happens to patients who got a quote and went quiet?

Many don't book anywhere soon, and some choose another practice, often because nobody followed up. A self-paid quote for implants, surgery or LASIK is a big decision, and "I need to think about it" is usually true.

A 30-day follow-up schedule, run by the treatment coordinator:

Day	Touch	Purpose
1	Call	Answer questions that came up at home
3	Email	Financing options and how payment plans work
7	Call	Check on timing; offer a second visit with the provider
14	Email	General recovery or timeline explainer for their type of treatment, no promised results
30	Call	Ask plainly whether to keep their spot or check back later

Then move them to a monthly general newsletter until they opt out. Someone should own the count of open quotes.

Will Google and AI assistants name our practice when patients ask?

Only if your pages answer the questions patients actually ask. BrightLocal's vendor survey, the Local Consumer Review Survey (February 11, 2026), found that use of ChatGPT and other AI tools for local recommendations rose from 6% to 45%. Patients ask "How much do full-arch implants cost in Cleveland?" An assistant may explain, then name a few practices.

We call this Search Essence Optimization: patients search to understand you, not just find you, so your pages should meet them where their questions are.

- [ ] One page per procedure with honest price ranges and the factors that move them
- [ ] Candidacy, recovery time and number of visits, explained plainly
- [ ] The treating clinician's name, credentials and a link to the state license lookup on every procedure page
- [ ] Financing explained in real terms, with no "starting at" bait

- [ ] Google Business Profile with the right primary category, services and current hours
- [ ] Before-and-after photos only with written patient permission, and no implied guarantee

Reviews matter too. BrightLocal found 68% of consumers require at least four stars and 47% won't use a business with fewer than 20 reviews. Google says local results depend mainly on relevance, distance and prominence, and you can't pay for a better local ranking. Ask every patient the same way, with no incentive tied to a positive review. The FTC rule finalized in August 2024 bans fake and AI-generated reviews, buying reviews and suppressing negative ones. In replies, never confirm the reviewer is a patient or mention treatment.

How do we track results without risking patient data?

Track to the booked consult, and keep everything after that inside your practice management system. HHS guidance on online tracking was narrowed when a federal court vacated the part covering unauthenticated public pages on June 20, 2024. Tracking on patient portals, scheduling pages and other pages that transmit protected health information is still covered, and any vendor that handles it needs a business associate agreement.

- [ ] List every form, call tracker, chat widget and booking tool on your site.
- [ ] Confirm a business associate agreement with every vendor that could see patient information.
- [ ] Remove ad and analytics pixels from scheduling and portal pages unless counsel has signed off.
- [ ] Report accepted treatment as counts by source in your own system.

Check with your healthcare attorney and your state board's advertising rules first. This is not legal advice.

Where do most owners get stuck?

> **Connecting the source to the accepted plan.** Getting each consult tagged with where it came from, without sending patient information to ad or analytics tools, takes careful setup on forms, phones and scheduling pages, and a recheck after every site change.

> **Keeping the five-minute promise.** The routine works until a busy Monday. Alerts, after-hours texts and consent records need to run on their own.

> **Being the practice the assistant names.** Cost and candidacy pages honest and specific enough for AI assistants to quote take steady work most practices can't spare.

If you want a second set of eyes on any of these, book a 25-minute exploration call through the short form at the end of this guide.

Sources

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Want a second set of eyes on your plan?

Book a free 25-minute exploration call. We will look at your numbers with you, name the first fixes in order, and tell you honestly if we are not the right fit.

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